Patient ID	: QLD25/1418	Sample ID	: 2502218
Name	**: MRS ARUNA**	Registration Date	: 05/02/2025 09:26 AM
Age/Gender	: 52 Yrs/Female	Report Date	: 05/02/2025 03:15 PM
Ref. Dr.	:	QLD25/1418	
Center	: Q LABS DIAGNOSTICS		

HAEMATOLOGY REPORT

Test Description	Result	Unit	Biological Reference Ranges
HBA1C/GLYCOCYLATED			
HbA1c Glycosilated Haemoglobin	**6.5**	%	4 - 6
Method: HPLC, NGSP certified			
Estimated Average Glucose :	140	mg/dL	

As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Action suggested: > 8.0 Age < 19 years Goal of therapy: <7.5

ADA criteria for correlation

HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126
7	154
8	183
9	212
10	240
11	269
12	298

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled .
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments: HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations.

Dr.SYEDA SALEHA FAREED MD PATHOLOGY
Authorised Signature

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BIOCHEMISTRY REPORT

Test Description	Result	Unit	Biological Reference Ranges
Fasting Plasma Glucose *Method: Hexokinase*	105	mg/dl	Normal : 74 - 106 Impaired GT : 107 - 125 Diabetic : >or=126

> or =126	Diabetic	Its a group of metabolic disorders characterized by a high blood sugar level over a prolonged period of time
101-125	Impaired GT	Impaired glucose tolerance means that blood glucose is raised beyond normal levels, but not high enough to warrant a diabetes diagnosis. With impaired glucose tolerance one can face a much greater risk of developing diabetes and cardiovascular disease.
70-100	Normal	The American Diabetes Association recommends a routine screening for type 2 diabetes. If the results are normal, the screening should be repeated every 3 years.

POST GLUCOSE BLOOD SUGAR (2HRS)

POST GLUCOSE BLOOD SUGAR *Method : Hexokinase*	135	mg/dL	70 - 140

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LIPID PROFILE

Cholesterol-Total *Method: Spectrophotometry*	**221**	mg/dL	< 200 - Desirable 200 - 239 -Boderline High > 240 - High
Triglycerides level *Method: Serum, Enzymatic, endpoint*	**284**	mg/dL	< 150 - Normal 150 - 199 -Boderline High 200 - 499 -High >500 Very -High
HDL Cholesterol *Method: Serum, Direct measure-PEG*	**44.2**	mg/dL	< 40 - Low > 40 - Normal
LDL Cholesterol *Method: Enzymatic selective protection*	120.00	mg/dL	< 100 - Optimal 100 - 129 - Near/Above Optimal 130 - 159 - Borderline high 160 - 189 - High > 190 - Very High
VLDL Cholesterol *Method: Serum, Enzymatic*	**56.80**	mg/dL	6 - 38
CHOL/HDL RATIO *Method: Serum, Enzymatic*	5.00		0.27 - 5.0
LDL/HDL RATIO *Method: Serum, Enzymatic*	2.71		2.5 - 3.5

NOTE

8-10 hours fasting sample is required

**** End of the report. ****
Please correlate clinically.

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